National Cancer Control Programme (NCCP) 00377 · clinician draft

CMF · oral cyclophosphamide

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1–14 · oral cyclophosphamide plus intravenous (IV) visits

Active medicines: Cyclophosphamide tablets Days 1–14 + Methotrexate and fluorouracil IV Days 1 and 8

Emetogenic risk: Moderate–high oral cyclophosphamide; IV agents low

Medicine	Dose / route	When
Ondansetron	8–16 mg total orally per day; prescriber selects exact dose	Start before systemic anticancer therapy (SACT) and continue daily through the oral-treatment course; confirm stop day NCCP oral Table 5. Review QT, constipation and hepatic dosing. Reconcile the same daily total on IV Days 1 and 8; do not add a duplicate IV-day order.

Additional phase-specific support

- Confirm allergy/interaction review, blood tests and the source-specific renal/hepatic assessment before prescribing.
- Confirm dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) status before first fluorouracil and review partial deficiency with the consultant.
- Confirm an individual hydration and bladder-symptom plan; adapt fluid advice to cardiac/renal restrictions.
- Review pleural effusion or ascites because methotrexate can accumulate in third-space fluid.
- NCCP oral guidance uses a daily 5-HT3 antagonist. Do not automatically apply the single-day parenteral high-risk/AC steroid schedule to this 14-day oral course.
- Specify rescue medicine and duration, and reconcile prophylaxis for two days after multiday SACT under the local plan.
- Cyclophosphamide tablets are Days 1–14 only; give clear missed/vomited-dose instructions. Methotrexate is IV in this regimen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00377 v6 · p3 · NCCP antiemetics v8, Table 1 pp4–5

NCCP oral antiemetic Table 5, p8 · Sources checked 2026-10-25 · SACTCheck v0.84.18 · Regimen-level drafts; prescriber and oncology-pharmacy verification required